

EMOTIONAL CANNIBALISM

SHAME IN ACTION

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INTRODUCTION

When Sami, the 21-year-old client described in this chapter, walked into my office there was a noteworthy discrepancy between what I saw—a young woman whose fashionable but conservative style conveyed her concern with propriety and appearances—and the referral notes I had just read, which included a laundry list of psychiatric diagnoses involving a high pitch of dys-regulated emotion. These included borderline personality disorder (due to self-cutting, intermittent suicidality and intense unstable relationships), major depression with psychotic features, binge eating disorder, body dysmorphic disorder and polysubstance abuse.

Sami's diagnoses indicated a whirl of conflict under her polished veneer and alerted me to the possibility that she had been traumatically shamed, probably in childhood. This chapter explores how interpersonal trauma can unleash the destructive force of shame into both external and internal relationships, morphing a dangerous event into a dangerous identity, which psychiatric symptoms—the behaviors of parts—are attempting to handle.

CHAPTER OVERVIEW

This chapter charts the movement of shame through external and internal relational systems, exploring motivations for shaming along with the consequences of feeling shamed. Because shaming can be intentional or unintentional, and the relational impact varies, I differentiate between the two. As Schore (2003) pointed out, unintentional shaming is an inevitable part of the relationship between children and any adult in the role of “socializing agent.” Differences in temperament between parent and child also

can give rise to unintentional shaming (Linehan, 1993). I focus here only on intentional shaming, either of oneself or someone else, and view it as an expeditious means of regulating shame in the short, though not the long run. The frame for exploring shaming is psychic multiplicity, or *parts*, and internal family systems therapy (IFS).

Intentional shaming, external or internal, has a damning message that feels highly personal and true to the recipient, as if something deplorable and permanent has been revealed. The ballpark message of shaming is that imperfection makes you bad, unacceptable and alone. Shaming someone else intentionally and cultivating a feeling of power in comparison to that belittled other can be quite effective in the short run at regulating shame. In contrast, self-directed shaming evokes shame and often precipitates symptomatic behavior like binge drinking and binge eating. But in either case, it is the thesis of this chapter that intentional shaming is commonly motivated by the urge to regulate the feeling of shame and that the target of attack will be determined by vulnerability and not (as it feels to the victim) worth.

The case example below illustrates Sami's chronic self-shaming, which followed from her father shaming her. In this session, the IFS protocol regulated Sami's shame with its generic evocation of compassion, calming a critical manager and several firefighters and laying the groundwork for healing her attachment injuries. But before illustrating Sami's experience in therapy I review the relationship between shame and psychopathology, note how researchers have decoded shame and look at how victim and perpetrator can become inextricable in the shame dynamic.

SHAME IN PSYCHOTHERAPY

Because the exquisite pain of feeling unacceptable, excluded and alone that typifies shame can be so toxic to current functioning, parts who experience it are often sealed off and left behind in time, with their management becoming a systemic enterprise that can last a lifetime. Therefore, with or without awareness, psychotherapists routinely navigate a host of symptoms associated with shame. The urges to withdraw, hide and avoid (the action tendencies of shame) show up in a wide variety of behaviors, including addiction, depression, anxiety, psychic numbing, cognitive attempts to undo reality, insensitivity to emotional and physical pain (both one's own and that of others) and dissociation (Dearing & Tangney, 2011; Lewis, 1971; Tangney & Dearing, 2002; Tracy, Robins, & Tangney, 2007). In converse to that the urge to regulate shame through attacking and blaming the self or others is also commonplace (Dearing & Tangney, 2011; Herbine-Blank, this volume; Tangney & Dearing, 2002; Tracy et al., 2007). Visible modes of self-attack that are often seen in mental health treatment include starving, cutting, burning, risk taking and suicide. More invisible forms include chronic self-criticism and self-loathing. Attacking others can involve a threatening tone and attitude, ostensible humor, contempt, being dismissive, entitled, disgusted or enraged; or it can involve actual assault up to the extremity of homicide (Gilligan, 1997). Shame is thus a force in much human conduct that is puzzling, alienating, dangerous and assigned either to the law or mental health professionals. We need effective strategies for treating it and, as this chapter illustrates, IFS is well designed to meet that need.

THE FUNCTION OF SHAME

Each human emotion is a directional compass with a true north derived from evolution, and true north for shame is conformity. Although conforming to the group norm has advantages, shame is a costly enforcer. Researchers agree that shame, like guilt, involves an internal audience giving a mental thumbs down to the self. In guilt this judgment is focused on behavior (*I did wrong*) while in shame it is global (*I am bad*) (Lewis, 1971; Tangney & Dearing, 2002; Tracy et al., 2007). The belief that one is being judged and that personal defects are being exposed launches the physical, emotional and cognitive cascade of the shame experience. This can include feeling queasy, heavy, shrunken, child-like, foolish, stupid, different, singular, alone, unwanted, un-lovable and worthless (Mills, 2005; Tangney & Dearing, 2002; Tracy et al., 2007). Relationally guilt and shame are opposites. Pure guilt promotes concern for others and a desire to repair (Tangney & Dearing, 2002; Tracy et al., 2007) whereas shame, along with guilt linked to shame (e.g., *I did wrong so I am bad*), promotes intense, negative self-focus, which is often followed by symptomatic behavior.

HISTORICAL VERSUS INSTRUMENTAL SHAME

Although the cyclical nature of the shame dynamic described in this chapter can make it appear that shame originates internally, children are not born ashamed. Therefore, this chapter takes the view that children are born with the capacity to feel shame but actually feel it due to experience. That is, shame is precipitated interpersonally, by shaming, and abuse or neglect can be intensely shaming.

In the description that follows, I call the range of intentional, interpersonal insults—from prejudice about physical, cultural and class differences to peer bullying, beatings and molestation—*historical shame*. Although historical shame is a passive, interpersonal experience, the internal dynamic that follows, *instrumental shaming* is active, self-generated and self-directed. Although the perspectives of victim and perpetrator are easily distinguished between historical shame and instrumental shaming, this distinction is soon lost once the victim incorporates the behavior of the perpetrator and transforms *a bad thing happened to me* into *I am bad*. The treatment goal of IFS is to help the client access enough Self-energy to reverse this sentence, returning *I am* to *it happened*.

Using the IFS lens, and looking at the motivation of parts, we can understand how a shamed individual turns to shaming someone else, or else picks up the habit of self-attack. In both cases, the victim becomes a perpetrator. The interpersonal perpetrator is generally a firefighter (a reactive protector) who is trying to regulate shame with externally focused anger, while the intrapsychic perpetrator is usually a manager (a proactive protector) trying to regulate shame with internally focused anger. Because self-criticism and self-attack evoke rather than suppress shame, critical managers tend to stimulate a wide variety of firefighter responses that involve either passive avoidance (like dissociation and denial) or the high-risk behaviors more likely to begin in adolescence (addictions, eating disorders,

self-harm, suicide, rage). Extreme firefighter behaviors torment the manager who, in turn, intensifies efforts to exert control with more internal shaming. Like an overactive immune system, this typical, escalating polarity between managers and firefighters can be very hazardous for the client's health.

THE CASE OF SAMI

Sami was born in California and had a pleasant childhood until the age of 5 years when her father, a prominent businessman, was arrested and put on trial for financial fraud. In response, he became violent at home—but only toward Sami. Her two older brothers were maintained in private boarding schools and her mother, an aloof woman with an active social life who spent little time at home, became even more distant during this crisis.

Thus Sami took the brunt of her father's rage. Although his behavior was morally reprehensible and traumatic for her, it was likely to have been instrumental for him. We can surmise that as he succumbed to self-loathing due to public humiliation, a firefighter stepped in to reboot his self-worth at the expense of the most vulnerable target in his vicinity. Once his equilibrium was restored, he did try to repair the relationship with Sami but by then she was a self-destructive teenager who used her mother's diet pills, restricted her calorie intake, purged after meals, had a violent boyfriend, binge drank with friends on weekends to the point of blacking out and destroyed her first car in a crash. In addition, when she was 17 she got pregnant. Her father and brothers insisted on an abortion while her mother, a devout Catholic, looked the other way.

Although Sami was a complete novice to the idea of psychic multiplicity, she took easily to *going inside*, Schwartz's (1995) term for paying attention to the subjective experience of having parts. In this session, a couple of months into treatment, she was just out of the hospital where she had gone after feeling suicidal about her performance in college and a planned holiday with her eldest brother, a graduate student at a prestigious business school. At the outset of the session she announced that she might need to return to the hospital.

"Why?" I asked.

"I have a part that wants me to believe my son is alive," she replied.

"How do you know?" I asked.

"It shows him to me," she said. "It wants me to take care of him. He's four now. But if I go there, I'll be crazy again."

In the IFS protocol the central goal is to foster a relationship inside the client between her *parts* and the core presence that differs from parts that Schwartz calls *Self*. IFS asserts that this core presence is sentient, motivated by love, exists in everyone and can be accessed in the emotional octave where feelings range from curiosity to compassion. Once the client has located and learned to stay in this affective interval while also being in relationship with her other feelings, even strong, negative ones, she has cracked the formula for attending safely to her subjective experience. But at this point in therapy

Sami was far from having the skill to accompany herself kindly, and my first goal, as she revealed a looming psychosis, was to help her access enough Self-energy to proceed safely to the second step of the IFS protocol, exploring the motivations of her activated parts.

So I asked Sami the key question for locating and helping parts to differentiate from the Self, “How do you feel toward the part who is showing the child to you?”

“Guilty,” she replied.

In IFS we stay with the target part, if possible, and ask other parts with strong feelings not to interfere. So I said, “Would the guilty part be willing to relax and let you help the part who brings your child?”

“OK,” Sami reported, “but it doesn’t want us to forget.”

“We may have to come back to it another week. If it’s worried that we have forgotten, will it please remind you?” I asked. Sami nodded. “How do you feel now toward the part who brings your child?” I said.

“I wish it would stop,” Sami said.

“Would the part who wants it to stop be willing to relax for a few minutes, too?”

“OK,” Sami replied.

“How do you feel toward it now?” I asked.

“I feel angry,” Sami said. “How can I live like this?”

“Would the angry part trust you and give you some space?” I said. After a pause, she nodded. “How do you feel now?” Because negotiating with protectors must continue in this way as long as they persist in doing whatever they do, the therapist must accommodate. However, once the client feels curious the action of the session can move to the next phase.

“I want to know why,” Sami said.

“Can you see the part who brings the child?” I asked.

“No,” Sami explained, “it stands behind, pushing.”

“Behind what?”

“Him.”

“What makes it decide to bring him to you?” I asked.

“It’s when the yelling gets louder,” she said.

“Who’s yelling?” I asked

“The part who tells me I’m a failure,” Sami said. “I’m not good enough. I’m ugly, fat, stupid. My brothers are the smart ones.”

I nodded, “The critic.” Critics are, as mentioned, marquee advertising for a historical, traumatic experience of being shamed, and they are extremely common. I went on, “If we could help this critic, would it be good for the part who wants you to believe your child is alive?”

“What do you mean, ‘help this critic’?” Sami asked suspiciously.

“I mean if the critic didn’t feel the need to criticize you anymore would this other part still feel a need to push him forward?”

“No,” Sami said. “But it doesn’t believe you can stop the critic.”

“I understand,” I said. “But will it let us try?”

Sami thought for a moment and then said, “OK.”

With this permission we shifted our attention to the critic and backtracked to the same basic questions that would help it differentiate from Sami's Self.

"Where do you notice the critic in your body?"

"It's not in my body," Sami replied. "It's around my body. It's like—you know the red Spider-Man suit with all the lines? It looks like that. My body dangles inside."

"How do you feel toward Spider-Man?" I asked.

"I'm glad he's there," Sami said.

"Ask the part who's glad he's there to give you room to be there too—just so you can get to know him," I said. Sami nodded. "How do you feel toward him now?"

"I understand him," she said.

"Let him know what you understand and see if you've got it right," I said.

"Yes," she said. "He protects me."

"Does he feel successful?" I asked.

"No," Sami observed. Then she frowned, "He's mad. He squeezes to punish me."

Reports of a part hurting the client's body illustrate that parts can and do affect physical experience and are an opportunity to negotiate more effective ways for the part to exert influence.

"Ask Spider-Man to stop doing that," I said. "He doesn't have to hurt you to get your attention. Tell him it's much more persuasive to be direct so you understand. Is he willing to stop?"

"For now," Sami reported.

Although managers such as Spider-Man are usually polarized with a firefighter, the part who is getting bullied (or, from the manager's point of view, protected) is a vulnerable, young exile. So I asked, "How old does Spider-Man think you are?"

"Five," Sami reported.

I said, "Let him know the five-year-old is another part and not you."

"He doesn't believe me," Sami said.

Because protective parts excel at escalating conflict but relax in response to curiosity, the best policy is to be curious. So I returned to the standard IFS question, "How do you feel toward him now?"

"Scared," Sami said.

"Is he still squeezing you?" I asked.

"No."

"Good. Then ask the scared part to relax and let you handle this."

"OK," she said.

"Now how do you feel toward him?"

"Interested," she said.

"Does he notice the difference?" I asked.

"Yes," she said. "He's puzzled."

"That you are not the five-year-old?"

"Yes."

"How old is he?"

"He won't tell."

Again, because opposition melts in the part's relationship to Self, I shift back to building this relationship. "Would you like to get to know Spider-Man better?"

"Yes," she said.

"Good. He has your attention, right? Remind him that he doesn't have to hurt you now," I said.

"He doesn't trust me."

"What does he want you to know about that?"

"He says where have I been?"

This question is a milestone in the relationship between the client's Self and her parts, so I repeated it. "He wants to know where you've been. What does he need to hear from you about that?"

"I didn't mean to leave him alone."

"Yes. Is he aware that when he or any other part takes you over, you can't be there with him?"

"He says why should he trust me now," Sami reported.

"He doesn't have to trust you without reason. Everybody has to earn trust," I said. "What does he want you to know about his job?"

"He protects me by saying those things to me."

Note that when Sami said, "He protects me," she had shifted back to the perspective of the 5-year-old girl.

"How's that going?" I asked.

"Not so good," Sami reported.

"He works hard but it's not going so good," I said.

"He's tired."

"No wonder."

"But he has to do it so he will."

"Really?" I asked. "Who is he responsible for?"

"Me," she said.

"The five-year-old?"

Sami paused before replying, "Yes actually. Her."

"Why does the five-year-old need help?" I asked.

"She's bad," Sami said.

"How did she get the idea that she's bad?" I asked.

"It happened to her," Sami said.

"Yes it did," I said.

"Yes it did," Sami repeated quietly.

"So of course she believed it," I said. "But tell Spider-Man that her father did all that to make himself feel better—she wasn't bad." Sami stopped speaking for more than a minute. "What's going on?" I finally asked.

"Spider-Man feels sad," she said. "He just wanted to make her good enough."

"By telling her she's not?"

"He's thinking," Sami reported.

"It's a puzzle, isn't it?" I said. "Shaming a little girl to protect her from feeling shame."

"He's confused," Sami reported.

“What’s the most confusing thing for him?”

“He wants to save her.”

“I can guess why he thought it might work. If I tell him, will he say if I’m right?”

“Yes.”

“OK. Spider-Man learned to treat you this way when he watched your father yelling and hitting you and then heard him say that he was doing it for you. But when Spider-Man does the same thing it scares the little girl, which scares those other parts who do wild things. And then her father and brothers are disappointed. And then Spider-Man feels even more desperate. He squeezes and yells more insults to make her better—”

Sami interrupted, “He hits me inside with words.”

“He hits you inside with words. And then what happens?”

“The little girl stands in the corner.”

“Does he hear us now?”

“Yes he’s listening.”

“Can he see that shaming and squeezing and hitting the little girl with words to keep her from feeling shame will never work?”

“But what can he do?”

“How do you feel toward him now, Sami?”

“I love him.”

“Does he know?” She nodded. I said, “Tell him there’s nothing wrong with the little girl and there never was—and I will show you how to take care of her so that Spider-Man is free to be himself. Would he like that?”

“Yes. But he says who is he?”

“When he has time to explore, he’ll find out.”

“OK. He likes that.”

We ended this session by checking back with the reactive part who was threatening to make Sami psychotic. It felt calmer and was, for the moment, willing to withdraw so Sami could stay out of the hospital and continue in therapy.

SUMMARY OF CASE EXAMPLE

Spider-Man’s relentless criticisms stirred up the exiled little girl’s fear and shame, which caused another protector to offer short-term comfort in the form of unreality, or psychosis. Although this part was reactive and came last in the chain of events, we noticed it first. In IFS we often work back from the most recent attempt at protection to an original injury. In this session, Sami noted an impending psychiatric emergency and from there we made our way to the shaming of Spider-Man before finally reaching the underlying cause, the shame and fear of the little girl.

Because this pattern of interactions between protectors is so characteristic and repetitive, I could predict that we would discover a manager provoking the behavior of the firefighter. Sami, however, was

only aware that she was about to be in serious trouble again. If she became psychotic, she would be hospitalized and put on medication, which would cause weight gain—a major precipitant for shame in her system—and would cause other firefighters to begin restricting, purging and planning suicide. Although this prospect terrified most of her system, the part pushing psychosis had no concern for long-term consequences. Hospital stay? Antipsychotics? Weight gain? Bulimia? Suicidality? Dropped credits? Family upset? Friends disappearing? Not my problem!

When a manager such as Spider-Man stimulates a firefighter to step in with extreme solutions and neither part is capable of unilateral de-escalation, ever more drastic consequences ensue. Even so, once Spider-Man differentiated enough to experience Sami's Self, he realized that she could help the little girl and he relaxed. This, in turn, decommissioned the firefighters who were gearing up for restricting, bingeing and purging and suicide. And we could continue in therapy toward the goal of unburdening the little girl and other exiles.

DISCUSSION

In sessions we hear protective parts frantically trying to undo what has been done to the client and sometimes what the client has done to others. The client's attachment injuries, and the attempts of her parts to prevent further injury, are the bread and butter of psychotherapy. As we have seen, therapy can track this historical experience back to the point where shaming judgments were accepted as identity (I am *bad*, *defective* and *alone* —and this is *all* I am) and where protective parts, in an effort to reverse these judgments, defensively incorporated the attitude of the perpetrator.

Ironically, we are capable of feeling shame for the same reason we can feel compassion: because the mind has the capacity to shift perspectives and move between mental vantage points. Yet without a concept of psychic multiplicity, this mental act of stepping out and looking back can be hard to account for. For example, when one is feeling shame, who is judging whom, and why? The answer in IFS is that individuals contain many subjects, an interactive multitude. Although shame is evoked for this multitude interpersonally, they maintain it intrapsychically. As a result, clients often come to us feeling that something is being done to them over which they have no control. Lacking a concept of how this can be true without someone else being involved, a sense of persecution, social exile and hopelessness are common.

In IFS, clients learn to enter into their psychic multiplicity and discover who is doing what to whom and why. And in contrast to the negative valence of shame, IFS teaches clients to go inside like contemplatives and meditators, those practitioners of perspective shifting who cultivate curiosity and compassion, the feeling states of Self that truly oppose (and are mutually exclusive with) shame.

CONCLUSIONS

Shaming is an extraordinarily dynamic phenomenon that loops from external to internal relationships and back, gathering strength like a hurricane that can blow the message *I am flawed and alone* through generations. Yet the content of shaming is a motivated fiction, often a shame regulation strategy and not an accurate communication about the worth or singular (and therefore shameful) nature of the individual being shamed. We are born imperfect but not unacceptable, unique or alone. We spend our lives embedded in relational systems, usually external, always internal. Clients who are guided into rather than away from their Self and their multiplicity of mind have the opportunity to undo, experientially, this fiction at the heart of shame.

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